



Thornbury Aged Care Services

Food, Nutrition and Dining Experience Policy

Menu planning, texture-modified diets, food safety and the dining experience

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Related standards	Standards 1, 5 and 6

This is a fictional document created to demonstrate document-summarisation software. The provider, facilities, personnel, incident references and document identifiers named in it do not exist. It is not a record of any real organisation and must not be relied on as compliance advice.

Document control

This document is controlled by the Head of Hospitality Services. Printed copies are uncontrolled and must be checked against the document register before use. Proposed amendments are submitted to the document owner and considered by the Clinical Governance Committee at its next scheduled meeting.

Version history

Version	Date	Author	Summary of change
2.0	22 Jun 2024	M. Castellano	Introduced the four-week seasonal menu cycle.
3.0	14 Mar 2025	M. Castellano	Adopted IDDSI terminology throughout, replacing local descriptors.
3.2	27 Nov 2025	A. Sharpe	Added the malnutrition screening pathway at Part 3.
3.4	9 Sep 2026	A. Sharpe	Rewritten against Standard 6; added dining experience requirements at Part 4.

Distribution and accessibility

The current version is published on the provider intranet and is available in hard copy at each nurses' station. Alternative formats, including large print and interpreted summaries, are provided on request to the document owner.

Audience	Method	Frequency
All care and clinical workers	Intranet and induction pack	On publication
Registered and enrolled nurses	Clinical handover briefing	On publication
Consumer Advisory Body	Emailed summary	Quarterly
Contracted and agency staff	Agency portal	Before first shift

Contents

Section	Title
Part 1	Purpose, principles and scope
Part 2	Menu planning and choice
Part 3	Nutrition, hydration and clinical risk
Part 4	The dining experience
Part 5	Food safety
Part 5A	Suppliers and contracted catering
Part 6	Governance and accountability for food, nutrition and dining
Part 7	Workforce competence and training
Part 8	Records, privacy and information management
Part 9	Monitoring, measurement and continuous improvement
Part 10	Risk management
Part 11	Review of this document
Part 12	Communication, engagement and supported decision-making
Part 13	Emergency, contingency and business continuity
Part 14	Subcontracted and external service providers
Part 15	Mapping to the Strengthened Aged Care Quality Standards
Part 16	Assurance, external review and regulatory engagement

Part 1 Purpose, principles and scope

Food is not catering. For a person living in residential aged care, meals structure the day, carry culture and memory, and are one of the few remaining sources of daily pleasure and choice. This document governs how Thornbury Aged Care Services plans, prepares and serves food, and how it makes dining an experience rather than a task to be completed.

The provider's position is that every person is entitled to food they recognise and want to eat, served at a temperature and texture that is safe for them, in company if they wish, without being rushed. Weight loss is treated as a clinical event, not an inevitability of ageing.

1.1 Scope

This document applies to all food and fluid provided by the service, including main meals, snacks, supplements, food brought in by families where the provider stores or reheats it, and food served at events. It applies to employed and contracted catering workers and to every worker who assists a person to eat.

1.2 Principles

- Food is offered, never imposed; a person may decline any meal or component.
- Choice is real, which means at least two hot options at lunch and dinner every day.
- Texture modification is a clinical prescription, not a catering convenience.
- Unplanned weight loss is investigated, not observed.
- Cultural, religious and personal food preferences are recorded and honoured.
- Mealtimes are protected from non-urgent clinical and administrative interruption.

Part 2 Menu planning and choice

The menu is planned on a four-week seasonal cycle by the Head of Hospitality Services with the accredited practising dietitian, and is reviewed by the Consumer Advisory Body before adoption.

2.1 Nutritional adequacy

Each menu cycle is assessed by the dietitian against the Australian Dietary Guidelines and the nutrient reference values for older adults, with particular attention to protein, calcium, fibre, vitamin D and fluid. The assessment is documented and retained. The current cycle's assessment must be completed by 30 November 2026.

2.2 Choice at every meal

Meal	Minimum choice	Service window
Breakfast	Two hot options, three cereals, fruit, toast, eggs cooked to order	7:00am to 9:30am
Morning tea	Two items, one savoury	10:00am to 10:45am
Lunch	Two hot mains, one salad or sandwich alternative, two desserts	12:00pm to 1:30pm
Afternoon tea	Two items, including a fresh option	2:30pm to 3:15pm
Dinner	Two hot mains, soup, one light alternative, dessert	5:00pm to 6:30pm
Supper and overnight	Sandwiches, fruit, yoghurt and hot drinks on request	Continuous

2.3 Access to food outside service times

Food and drink must be available 24 hours a day. A person who is hungry at 3am is entitled to a meal, not a biscuit. Night staff have access to a stocked pantry and are trained to prepare a light hot meal. Availability outside service times is checked at each night audit.

2.4 Cultural and personal preferences

Each person's food preferences, cultural and religious requirements, allergies and intolerances are recorded on entry to care, confirmed at every care plan review, and made visible to catering workers through the dietary profile. A preference recorded but not delivered is a complaint and is treated as one.

Part 3 Nutrition, hydration and clinical risk

Malnutrition and dehydration are common, serious and largely preventable. They are identified through routine screening rather than by waiting for visible decline.

3.1 Screening and monitoring

Requirement	Frequency	Responsible
Validated malnutrition screening on entry to care	Within 7 days of entry	Registered nurse
Weight recorded	Monthly, or weekly where risk is identified	Care worker
Rescreen for malnutrition risk	Every 3 months	Registered nurse
Dietitian referral following unplanned weight loss	Within 7 days of identification	Registered nurse
Fluid balance monitoring where dehydration risk identified	Each shift	Care worker
Nutrition care plan review	Every 6 months, or on change	Registered nurse and dietitian

3.2 Unplanned weight loss

Unplanned loss of 3 kilograms or more, or 5 per cent of body weight, over three months triggers a clinical review, a dietitian referral within 7 days, and a documented nutrition care plan. The person's supporter is informed. Weight loss is never recorded and left without action.

3.3 Texture-modified diets and thickened fluids

Texture modification is prescribed by a speech pathologist or medical practitioner following assessment, and is recorded using International Dysphagia Diet Standardisation Initiative levels. Local descriptors such as 'soft diet' or 'minced' must not be used on any chart, whiteboard or tray card. A worker who is unsure of a person's level must check the dietary profile before serving; assumption is not acceptable.

3.4 Assistance to eat

A person requiring assistance is supported by a worker who sits at eye level, unhurried, offering one mouthful at a time and allowing the person to set the pace. Assistance is never provided by a worker standing over a person, and never by more than one person at a time. Mealtime assistance requirements are recorded in the care plan and communicated at handover.

Part 4 The dining experience

Standard 6 requires more than nutrition. The dining room should look and feel like a place people choose to eat, and the meal should be the centre of attention rather than an interruption to the routine of the shift.

4.1 Protected mealtimes

Non-urgent clinical rounds, medication administration where it can be timed differently, cleaning, maintenance and administrative visits do not occur during meal service. Urgent clinical care always takes precedence. Protected mealtimes are monitored by observational audit each month.

4.2 The dining environment

- Tables set with cloths, crockery and cutlery, not disposable service, unless infection control requires otherwise.
- Meals plated at or near the point of service rather than delivered pre-plated and cold.
- Television off and noise minimised during service.
- Adequate lighting, contrasting crockery for people with visual impairment, and adaptive cutlery where needed.
- Workers seated with residents where staffing permits, encouraging conversation.

4.3 Food temperature and presentation

Hot food is served at or above 65 degrees Celsius at the point of service and cold food at or below 5 degrees. Temperatures are checked and recorded at every service. Texture-modified meals are presented in moulded or piped form so that each component is distinguishable; a plate of undifferentiated puree is not acceptable presentation.

4.4 Feedback on food

Food is the most common subject of feedback in residential aged care and is treated as valuable information. A food focus group meets monthly with the Head of Hospitality Services, and menu changes

arising from feedback are reported back to residents within one cycle. The next menu review incorporating resident feedback is due by 31 March 2027.

Part 5 Food safety

The provider operates a documented food safety program under Standard 3.2.2A of the Food Standards Code, appropriate to a service preparing food for a highly vulnerable population.

5.1 Temperature control

Cold food is stored at or below 5 degrees Celsius and hot food held at or above 60 degrees. Cooked food that is to be cooled must pass from 60 to 21 degrees within two hours and from 21 to 5 degrees within a further four hours. All temperature records are retained for two years and reviewed weekly.

5.2 Cleaning, hygiene and pest control

Cleaning schedules are documented, signed and verified by the Head of Hospitality Services weekly. Workers with vomiting, diarrhoea or an infectious skin condition must not handle food and must not return until 48 hours after symptoms resolve. Pest inspections occur quarterly, and the next is due by 31 December 2026.

5.3 Food brought in by families

Families are welcome to bring food. Where the provider stores or reheats it, it must be labelled with the person's name and the date, refrigerated within two hours, and discarded after 48 hours. Workers must check the person's dietary profile before serving food brought in, particularly where a texture modification or allergy is recorded.

5.4 Recall and external audit

The provider acts on food recall notices within 24 hours of publication, removing affected stock and recording the action. An independent food safety audit is conducted annually; the next external audit falls due by 30 June 2027.

Part 5A Suppliers and contracted catering

Where food is prepared by a contractor or supplied by an external kitchen, the provider remains accountable for what is served.

5A.1 Supplier requirements

- Current food safety certification, verified annually.
- Ability to supply every IDDSI level the service requires.
- Agreement to the provider's temperature, labelling and traceability requirements.
- Participation in the provider's menu review and food focus group.
- Acceptance of the provider's right to audit on reasonable notice.

5A.2 Supplier review

Each food supplier is reviewed annually against complaints, temperature compliance, delivery reliability and audit findings. The current supplier review cycle concludes on 30 April 2027.

Part 6 Governance and accountability for food, nutrition and dining

The Board of Thornbury Aged Care Services holds ultimate accountability for food, nutrition and dining and discharges that accountability through the Food and Nutrition Committee. The Board receives a standing report at each ordinary meeting covering performance against the indicators set out in Part 9, together with any matter the Head of Hospitality Services considers requires the Board's attention before the next reporting cycle.

The Food and Nutrition Committee meets monthly. Its quorum is four members and must include at least one registered nurse and one member independent of the operational management line. Minutes are retained for seven years and are made available to the Aged Care Quality and Safety Commission on request.

6.1 Allocation of responsibilities

Responsibility for the requirements of this document is allocated as set out below. Allocation of a responsibility does not relieve any worker of their own professional and legal obligations.

Role	Responsibility
Board	Receives food and nutrition indicators quarterly, including unplanned weight loss rates.
Head of Hospitality Services	Owens this document; plans menus; verifies food safety and temperature records.
Accredited practising dietitian	Assesses menu adequacy; reviews people at nutritional risk; advises on supplements.
Speech pathologist	Assesses swallowing; prescribes IDDSI levels; reviews texture modification.
Registered nurses	Complete malnutrition screening; act on weight loss; maintain nutrition care plans.
Care workers	Record weights and intake; provide mealtime assistance; report refusals and difficulty swallowing.
Catering workers	Prepare food to the prescribed texture; record temperatures; follow the food safety program.

6.2 Delegation

The Head of Hospitality Services may delegate an operational function under this document in writing, recording the scope and duration of the delegation in the delegations register. A delegation must not exceed twelve months without review, and the delegate must hold the qualifications and current registration the function requires. The Head of Hospitality Services remains accountable for a delegated function.

6.3 Conflicts of interest

A worker who has a personal, financial or familial interest that could reasonably be perceived to influence a decision made under this document must declare that interest to the Head of Hospitality Services before taking part in the decision. Declared interests are recorded in the conflicts register and reviewed by the Food and Nutrition Committee each quarter.

Part 7 Workforce competence and training

No worker may perform a function governed by this document until they have completed the training set out in this Part and been assessed as competent. The Head of Hospitality Services maintains a training matrix mapping every role to its mandatory modules, and the workforce management system prevents rostering a worker to a function for which their competency has lapsed.

Module	Audience	Frequency	Assessment
Food safety and hygiene	All catering workers	Annually	Written assessment and observation
IDDSI levels and texture modification	Catering and care workers	Annually	Practical assessment
Assisting a person to eat safely	All care workers	Annually	Direct observation
Recognising choking and dysphagia	All workers	Annually	Scenario assessment
Malnutrition screening	Registered and enrolled nurses	Annually	Tool-based assessment

7.1 Induction

New workers complete the food, nutrition and dining component of induction before their first unsupervised shift. Induction covers this document, the escalation pathway in Part 5, and the worker's obligation to report. A worker who has not completed induction may work only under the direct supervision of a competent worker.

7.2 Competency assessment and remediation

Competency is assessed by direct observation against the criteria at Appendix B, not by attendance alone. A worker assessed as not yet competent is placed on a supervised practice plan for a defined period, reassessed at the end of that period, and stood down from the relevant function if the second assessment is not satisfactory.

The Head of Hospitality Services reports training completion rates to the Food and Nutrition Committee monthly. Where completion for any mandatory module falls below 95 per cent, the Head of Hospitality Services must provide the Food and Nutrition Committee with a written recovery plan at the next meeting.

Part 8 Records, privacy and information management

Records created under this document form part of the care record and are managed in accordance with the provider's Records Management Policy and the Privacy Act 1988 (Cth). Entries must be contemporaneous, factual, legible and attributed to an identified author, and must not be altered after the fact except by a dated

and attributed addendum.

8.1 Retention

Record	Minimum retention	Custodian
Food temperature records	2 years	Head of Hospitality Services
Menu nutritional assessments	5 years	Head of Hospitality Services
Weight and nutrition monitoring records	Life of the care record	Health Information Manager
Cleaning and pest control records	2 years	Head of Hospitality Services
Supplier certification and audit reports	7 years	Procurement Manager

8.2 Access and disclosure

An individual, or their supporter where authorised, may access records held about them on written request. Requests are actioned within 30 days. Personal information is disclosed outside the provider only with consent, or where disclosure is required or authorised by law, and every such disclosure is recorded in the disclosure register.

8.3 Information security

Access to the clinical information system is role-based and individually credentialled. Shared logins are prohibited. Access is revoked on the day a worker's engagement ends, and access logs are reviewed quarterly for anomalous patterns. A suspected data breach is escalated immediately under the Notifiable Data Breaches scheme.

Part 9 Monitoring, measurement and continuous improvement

Performance under this document is measured against the indicators below. The Head of Hospitality Services reports results to the Food and Nutrition Committee monthly and to the Board quarterly. An indicator that falls outside its target for two consecutive months triggers a documented improvement action with a named owner and a due date.

Indicator	Target	Source	Frequency
Residents with unplanned weight loss in the quarter	below 8%	Quality indicator extract	Quarterly
Malnutrition screening completed within 7 days of entry	100%	Clinical audit	Monthly
Dietitian referral within 7 days of identified weight loss	at least 95%	Referral log	Monthly
Hot food served at or above 65 degrees	at least 98%	Temperature records	Weekly
Protected mealtimes observed without interruption	at least 90%	Observational audit	Monthly
Resident satisfaction with food	at least 80%	Consumer experience survey	Annually

9.1 Internal audit

The Head of Hospitality Services conducts a documented internal audit of compliance with this document at least twice each year, using the tool at Appendix C. Findings are rated as conformity, opportunity for improvement, minor non-conformity or major non-conformity. A major non-conformity must be reported to the Food and Nutrition Committee within five business days of identification.

9.2 Continuous improvement register

Improvement actions arising from audits, incidents, complaints, feedback and consumer advisory input are recorded in the continuous improvement register with an owner, a due date and an evidence field. The register is reviewed at each committee meeting and overdue items are escalated to the Board.

9.3 Consumer and workforce input

The provider seeks the views of individuals receiving care, their supporters and the workforce on the operation of this document at least annually, through the consumer experience survey, resident and relative meetings and workforce feedback channels. Results and the provider's response are published in the quarterly quality report.

Part 10 Risk management

Risks to the delivery of the requirements in this document are assessed using the enterprise risk matrix, which rates a risk on the likelihood of occurrence against the consequence for the individual receiving care. Risks rated high or extreme are recorded on the strategic risk register and reviewed by the Board each quarter.

Risk	Rating	Existing controls	Treatment
A person served the wrong texture level chokes	High	IDDSI terminology only; dietary profile at point of service	Tray card redesign with IDDSI level and photo by 28 February 2027
Unplanned weight loss recorded but not acted on	High	Automatic referral trigger at 3kg or 5%	Weekly weight exception report from 1 December 2026
Food-borne illness outbreak	High	Documented food safety program; temperature control	External food safety audit by 30 June 2027
Food brought in by family conflicts with a modified diet	Medium	Dietary profile check before serving	Family information sheet issued by 31 January 2027
Dining experience degraded by task-focused service	Medium	Protected mealtimes; observational audit	Monthly audit results to the committee
Night-time food access not available in practice	Medium	Stocked pantry; night staff trained	Night audit each month

10.1 Dignity of risk

The provider supports each individual to make their own decisions, including decisions that carry risk. Where an individual chooses a course of action a clinician considers unsafe, the discussion, the information given, the individual's decision and any agreed mitigations are documented in the care plan and reviewed at

each care plan review. Supporting a person's choice is not a failure of care.

Part 11 Review of this document

This document is reviewed at least every two years, and sooner where a legislative change, a major non-conformity, a serious incident, a coronial finding or a substantive change to practice makes review necessary. The next scheduled review falls due on 1 November 2028.

The review is coordinated by the Head of Hospitality Services and must involve at least one registered nurse, one worker who applies the document in daily practice, and one consumer representative drawn from the Consumer Advisory Body. The reviewed document is approved by the Food and Nutrition Committee and, where the change is material, by the Board.

11.1 Related documents

- TA-CLIN-004 Swallowing and Dysphagia Management Procedure
- TA-CLIN-009 Weight Monitoring and Nutrition Care Procedure
- TA-OPS-002 Food Safety Program
- TA-CORP-003 Complaints, Feedback and Open Disclosure Policy
- TA-CLIN-016 Infection Prevention and Control Policy

11.2 Legislative and standards context

- Aged Care Act 2024 (Cth) and the Aged Care Rules made under it
- Strengthened Aged Care Quality Standards, effective 1 November 2025
- Privacy Act 1988 (Cth) and the Australian Privacy Principles
- Work Health and Safety Act 2011 (Cth) and corresponding state legislation
- Food Standards Australia New Zealand Food Standards Code, Standard 3.2.2A

Part 12 Communication, engagement and supported decision-making

Every person receiving care from Thornbury Aged Care Services has the right to be told, in a way they understand, what is happening in their care and why. Information provided under this document must be offered in the person's preferred language and format, and must be repeated or restated where comprehension is uncertain. Reliance on a nod or a signature is not evidence of understanding.

12.1 Interpreters and communication support

An accredited interpreter must be engaged where a person's preferred language is not English and the discussion concerns consent, risk, a change to care, an incident affecting them or a complaint. Family members must not be used as interpreters for these discussions except in an emergency, and where that occurs the reason is recorded and the discussion repeated with an accredited interpreter within two business days.

12.2 Supported decision-making

The provider presumes that every person has decision-making capacity. Where capacity is in question, the assessment must be decision-specific, time-specific and documented, and must not be inferred from a diagnosis, age or communication difficulty. A person's supporter assists the person to make and communicate their own decision; a substitute decision-maker acts only where the person cannot make the decision even with support.

12.3 Engagement with supporters and representatives

The provider maintains a current record of each person's nominated supporters and representatives, including the scope of any authority held and the instrument granting it. The record is confirmed with the person at each care plan review and whenever the person's circumstances change.

12.4 Culturally safe practice

Care is delivered in a way that is safe for the person's culture, spirituality, gender, sexuality and life experience. For Aboriginal and Torres Strait Islander people, the provider offers connection to community-controlled health services and to family and community, and records the person's wishes about who is involved in decisions about their care.

Part 13 Emergency, contingency and business continuity

The requirements of this document continue to apply during an emergency. Where an emergency makes ordinary compliance impossible, the provider must record what was done instead, why, and for how long, and must return to ordinary practice as soon as the emergency permits.

13.1 Planned system outages

Where the clinical information system is unavailable, workers record required information on the approved paper forms and enter it into the system within 24 hours of the system being restored. The paper record is retained and scanned into the care record; it is not destroyed after entry.

13.2 Surge, outbreak and staffing shortfall

During an infectious disease outbreak or a staffing shortfall, the Head of Hospitality Services may authorise temporary variations to non-statutory requirements of this document. Statutory obligations, including notification timeframes, cannot be varied. Every temporary variation is recorded in the variations log with the authorising officer, the reason, the start date and the review date, and is reported to the next committee meeting.

13.3 Evacuation and relocation

Where a person is relocated in an emergency, the receiving service must be given the person's current care plan, medication chart, advance care directive and supporter contact details before or with the person. A relocation is not complete until a handover has been given to and acknowledged by a named clinician at the receiving service.

Part 14 Subcontracted and external service providers

The provider remains fully accountable for care delivered on its behalf by a subcontractor, agency or external practitioner. Engaging another organisation transfers work; it does not transfer accountability.

14.1 Contractual requirements

- Every service agreement must require compliance with this document by name.
- Every agreement must require the subcontractor to report incidents to the provider immediately, and in any event within four hours of becoming aware.
- Every agreement must permit the provider to audit the subcontractor's compliance on reasonable notice.
- Every agreement must require workers supplied to hold current screening and the qualifications the role requires.
- Every agreement must be reviewed before renewal, and no agreement may be renewed where a material non-conformity remains unresolved.

14.2 Monitoring subcontractor performance

The provider reviews the performance of each subcontractor at least annually against the indicators at Part 9, and more frequently where a concern has been identified. The review considers incidents, complaints, workforce screening currency and audit findings. The next scheduled subcontractor review cycle concludes on 31 March 2027.

14.3 Agency and locum workers

An agency worker may not commence a shift until the provider has sighted evidence of current registration where the role requires it, a current worker screening clearance, and completion of the provider's orientation. The orientation attestation is retained for the duration of the engagement and for seven years afterwards.

Part 15 Mapping to the Strengthened Aged Care Quality Standards

This Part records how the requirements of this document give effect to the Strengthened Aged Care Quality Standards. It is used as the starting point for self-assessment and for evidence gathering before a quality audit.

Standard	Outcome	How this document gives effect to it
Standard 6	Food and nutrition	Parts 2 and 3 govern menu adequacy, choice and nutritional risk.
Standard 6	Dining experience	Part 4 requires protected mealtimes and a dining environment.
Standard 1	Dignity and choice	2.4 requires cultural and personal preferences to be honoured.
Standard 5	Clinical care	3.3 makes texture modification a clinical prescription.
Standard 3	Care and services	3.1 sets screening, weighing and review requirements.
Standard 2	The organisation	Part 9 reports weight loss and satisfaction indicators to the Board.

15.1 Evidence for audit

Evidence supporting each mapped outcome is maintained in the evidence library and indexed against the outcome number. Evidence must be current, dated and attributable, and must demonstrate practice rather than intention: a policy document alone is not evidence that the practice occurs.

15.2 Self-assessment

The Head of Hospitality Services completes a self-assessment against the mapped outcomes at least annually and before any planned quality audit. The self-assessment is endorsed by the Food and Nutrition Committee and reported to the Board. The next self-assessment is due by 30 April 2027.

Part 16 Assurance, external review and regulatory engagement

Assurance over this document operates on three lines: the workers and managers who apply it day to day, the Food and Nutrition Committee which monitors and challenges performance, and independent review commissioned by the Board. Each line is documented and none substitutes for another.

16.1 Independent review

The Board commissions an independent review of compliance with this document at least every three years, or sooner following a major non-conformity or a serious incident. The reviewer must not have been involved in writing or operating the document. The report goes to the Board unaltered, together with management's response. The next independent review must be completed by 30 September 2027.

16.2 Engagement with the Commission

The provider cooperates fully and promptly with the Aged Care Quality and Safety Commission. Requests for information are acknowledged within one business day and answered within the period specified, or an extension is sought in writing before the period expires. A worker approached directly by the Commission may speak with it freely; the provider must not require a worker to obtain permission first.

16.3 Responding to findings

Where a regulator or reviewer identifies a deficiency, the provider prepares a written response within 14 days setting out what will change, who is accountable, when it will be done and how effectiveness will be measured. Progress is reported to the Board monthly until every action is closed and verified.

16.4 Sanctions and notifiable changes

Where a sanction, a notice to agree, or a notice of non-compliance is issued, the Head of Hospitality Services must notify the Board Chair on the day it is received, and the provider must tell affected individuals and their supporters what it means for their care within five business days.

Appendix A — Definitions

The following definitions apply throughout this document.

Term	Definition
IDDSI	The International Dysphagia Diet Standardisation Initiative framework describing food textures and drink thicknesses.
Texture-modified diet	Food altered in texture on clinical prescription to reduce the risk of choking or aspiration.
Malnutrition screening	Use of a validated tool to identify a person at risk of malnutrition.
Unplanned weight loss	Loss of 3 kilograms or 5 per cent of body weight over three months without planned intent.
Protected mealtime	A period during which non-urgent activity is deferred so that people can eat without interruption.
Dietary profile	The record of a person's preferences, allergies, cultural requirements and prescribed texture level.

Appendix B — Competency assessment criteria

A worker is assessed as competent when they demonstrate each criterion below by direct observation in the workplace. The assessor records the date, the criterion, the outcome and any remediation required.

Criterion	Evidence sought	Outcome
Identifies a person's IDDSI level before serving	Observed checking the dietary profile at the point of service	Competent / Not yet
Prepares food to a prescribed IDDSI level	Practical assessment against the IDDSI flow and fork tests	Competent / Not yet
Assists a person to eat safely	Observed seated, unhurried, correct positioning	Competent / Not yet
Records and escalates weight loss	Correctly identifies the referral trigger	Competent / Not yet
Responds to a choking event	Demonstrates the response in simulation	Competent / Not yet

Appendix C — Internal audit tool

This tool is completed at each internal audit. Each question is answered yes, no or not applicable, with a comment recording the evidence examined. A 'no' answer requires a corrective action with an owner and a due date.

#	Audit question	Evidence examined
1	Was malnutrition screening completed within 7 days of entry for every person in the sample?	Clinical records
2	Where unplanned weight loss occurred, was a dietitian referral made within 7 days?	Referral log
3	Do all charts and tray cards use IDDSI terminology rather than local descriptors?	Charts, tray cards
4	Were food temperatures recorded at every service and within range?	Temperature records
5	Were protected mealtimes observed during the audit period?	Observational audit
6	Is food genuinely available outside service times, including overnight?	Night audit
7	Is the menu cycle supported by a current dietitian assessment?	Menu assessment
8	Was food brought in by families labelled, dated and discarded within 48 hours?	Refrigerator inspection

Appendix I — Abbreviations

The following abbreviations are used in this document and in the records it requires. Abbreviations must not be used in clinical entries where the meaning could be ambiguous.

Abbreviation	Meaning
ACQSC	Aged Care Quality and Safety Commission
ATAGI	Australian Technical Advisory Group on Immunisation
CGC	Clinical Governance Committee
CEO	Chief Executive Officer
EN	Enrolled nurse
IDDSI	International Dysphagia Diet Standardisation Initiative
MDT	Multidisciplinary team
PPE	Personal protective equipment
PRN	Pro re nata, meaning as required
RMMR	Residential Medication Management Review
RN	Registered nurse
SDM	Substitute decision-maker
SIRS	Serious Incident Response Scheme
WHS	Work health and safety

Appendix J — Worker acknowledgement

Every worker to whom this document applies must read it and acknowledge that they have done so. Acknowledgement records that the worker has read the document; it does not replace the competency assessment at Appendix B.

Acknowledgements are recorded in the learning management system. A worker who has not acknowledged the current version within 30 days of its publication is reported to their line manager, and a worker who has not acknowledged it within 60 days may not be rostered to a function this document governs until they do.

Acknowledgement statement

I confirm that I have read and understood this document, that I have had the opportunity to ask questions about it, and that I understand my obligations under it. I understand that where I am unsure how this document applies to a situation, I am expected to ask before acting rather than to assume.

Name	Role	Signature	Date

Completed acknowledgement sheets are returned to the document owner and retained for the life of the version plus seven years.

Appendix H — Implementation plan for the current version

The following milestones apply to the implementation of this version. Milestones are tracked by the document owner and reported to the committee monthly until all are closed.

Milestone	Owner	Due
Publish version 3.4 and brief catering and care workers	Document owner	1 Nov 2026
Complete the dietitian assessment of the current menu cycle	Head of Hospitality Services	30 Nov 2026
Commence weekly weight exception reporting	Quality Manager	1 Dec 2026
Complete IDDSI refresher training for all catering workers	Clinical Educator	31 Jan 2027
Roll out redesigned tray cards	Head of Hospitality Services	28 Feb 2027
Report implementation completion to the Board	Head of Hospitality Services	30 Apr 2027

Appendix E — Escalation matrix

This matrix sets out who must be told, and how quickly, when a matter arises under this document. Where two rows could apply, the shorter timeframe governs.

Trigger	Escalate to	Within
Choking or suspected aspiration	Registered nurse in charge, then 000 if required	Immediately
A person served the wrong texture level	Registered nurse and incident process	Immediately
Suspected food-borne illness in two or more people	Head of Hospitality Services and Infection Control Lead	Immediately
Unplanned weight loss meeting the trigger	Registered nurse, then dietitian	Within 7 days
Hot food below 60 degrees at service	Head of Hospitality Services	Same service
Food recall notice published	Head of Hospitality Services	Within 24 hours

Appendix F — Forms and templates

The following forms support this document. Forms are version-controlled and must be obtained from the intranet rather than copied from a local drive.

Form	Purpose	Retained by
TA-F-005a Dietary profile	Records preferences, allergies, cultural needs and IDDSI level	Registered nurse
TA-F-005b Temperature record	Records hot and cold temperatures at each service	Catering worker
TA-F-005c Weight monitoring chart	Records monthly weights and calculates change	Care worker
TA-F-005d Nutrition care plan	Records dietitian recommendations and monitoring	Registered nurse
TA-F-005e Mealtime observation audit	Records protected mealtime and dining environment observations	Quality Manager

Appendix G — Frequently asked questions

These questions arose during consultation on the current version and are reproduced with the answers given.

Question	Answer
A resident wants a fried breakfast every day against dietitian advice. What do we do?	Serve it. The person's choice prevails. Record the discussion, ensure they understand the advice, and note the decision in the care plan under dignity of risk.
The tray card says 'soft diet'. Is that acceptable?	No. Local descriptors are prohibited because they mean different things to different workers. Check the dietary profile for the IDDSI level before serving.
A resident asks for a meal at 2am. What is available?	A light hot meal. Night staff have pantry access and training to prepare one. Food must be available 24 hours a day.
A family brings a home-cooked meal for a resident on thickened fluids. Can we serve it?	Check the dietary profile first and speak with the registered nurse. Food brought in must still meet the person's prescribed texture and fluid requirements.
Do we have to stop medication rounds during meals?	Where the timing can safely be changed, yes. Protected mealtimes defer non-urgent activity. Time-critical medicines and urgent clinical care always continue.

Appendix D — Improvement actions arising from the current review

The following actions were agreed at the most recent review of this document and are tracked in the continuous improvement register until closed.

Ref	Action	Responsible	Due
F1	Complete the dietitian assessment of the current menu cycle	Head of Hospitality Services	30 Nov 2026
F2	Introduce a weekly weight exception report	Quality Manager	1 Dec 2026
F3	Quarterly pest inspection	Facilities Manager	31 Dec 2026
F4	Issue a family information sheet on food brought in	Head of Hospitality Services	31 Jan 2027
F5	Redesign tray cards to show IDDSI level and photograph	Head of Hospitality Services	28 Feb 2027
F6	Complete the independent food safety audit	External auditor	30 Jun 2027